

CLINICAL FINDINGS

Sub-Types of Melanoma

SUPERFICIAL SPREADING MELANOMA

Superficial spreading melanoma (SSM) is the most common subtype, accounting for approximately 70 percent of all cutaneous melanomas. It is most commonly diagnosed in the fourth and fifth decades, typically arising on intermittently sun-exposed areas—most frequently on the lower extremity in women and the upper back in men.

Its classic clinical appearance aligns well with the ABCD criteria (see *Making a Diagnosis*), featuring irregular borders and irregular pigmentation. However, it may also present subtly as a focal area of darkening within a pre-existing nevus. The clinical spectrum of SSM is broad. While varying shades of brown are typical of most melanocytic lesions, melanomas may also exhibit dark brown to black, blue-grey, pink, red, or grey-white hues—the latter possibly indicating regression.

SSM is the melanoma subtype most commonly associated with pre-existing nevi. The clinical history often involves a lesion that slowly changes over months to years. It may be mistaken for an atypical nevus or seborrheic keratosis.

NODULAR MELANOMA

Nodular melanoma (NM) is the second most common subtype, accounting for approximately 15 to 30 percent of all melanomas. The median age of onset is 53 years, with the trunk being the most frequent site.

NM is characterized by rapid evolution, often developing over several weeks to months. It typically lacks an apparent radial growth phase and more commonly arises *de novo* rather than from a pre-existing nevus.

Clinically, NM usually presents as a uniformly dark blue-black or bluish-red raised lesion. However, about 5 percent are amelanotic. A significant proportion of thick melanomas are of the nodular type.

Early lesions may lack asymmetry but often have irregular borders and are larger than 6 mm in diameter. Amelanotic lesions can be mistaken for basal cell carcinoma, pyogenic granuloma, or hemangioma, while pigmented forms may resemble blue nevi or pigmented basal cell carcinomas.

LENTIGO MALIGNA AND LENTIGO MALIGNA MELANOMA

Lentigo maligna (LM) is a subtype of melanoma *in situ* with a prolonged radial growth phase that may progress over time to invasive lentigo maligna melanoma (LMM). Invasive LMM accounts for 10 to 15 percent of cutaneous melanomas.

LM and LMM are most commonly diagnosed in the seventh to eighth decades of life—later than other melanoma subtypes—and are uncommon before the age of 40.

These lesions typically occur on chronically sun-exposed areas, especially the face, including the cheeks and nose. Other common sites include the neck, scalp, and ears.